ILLUSTRATIVE SAMPLE · PREPARED TO DEMONSTRATE FORMAT AND METHOD · PROGRAM FIGURES ARE NOT ACTUAL DATA

GARRETT LEE SMITH SUICIDE PREVENTION PROGRAM · SAMHSA

Quarterly Evaluation Report

Quarter 2, Budget Year 1 — January 1 to March 31, 2027

PREPARED FOR
Department of Wellness and Prevention, Tanana Chiefs Conference Health Services
200 First Avenue, Fairbanks, Alaska 99701

PREPARED BY
ChiResearch Technologies, LLC · 6828 Flagstaff St, Hyattsville, MD 20785
Wilfred C. Ikejiofor, Evaluation Lead · wilfred@chiresearchai.com · 410-929-2170

DISTRIBUTION
Department Director · Wellness and Prevention program staff · SAMHSA project officer

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April 14, 2027 — within 14 days of quarter close

What this document is. This is a sample quarterly report produced by ChiResearch Technologies, LLC to show the Department exactly what it would receive each quarter under this contract: the format, the level of detail, the data-quality disclosure, and the audit trail from every figure to its source.

What is real and what is not. The five performance indicators, their definitions and the reporting cadence are taken from the published FY2026 Garrett Lee Smith State/Tribal notice of funding opportunity. External data sources are cited accurately, with their real coverage and limitations. **All program activity figures are illustrative** — they describe no real participants, no real services and no real outcomes, and they are marked as illustrative wherever they appear.

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Quarter 2 at a glance

> **Screening came online this quarter and immediately became the program's largest activity.** Training continued at roughly the Quarter 1 pace. The measure that needs attention is not any single count — it is the gap between referral and service receipt, where 12 of 31 referred individuals have no recorded service contact. That gap, not the volume, is what will shape the annual report.

What moved

- Screening for suicidal ideation began in January after instrumentation completed in Quarter 1. *214 individuals screened* across five sub-regions.
- Suicide prevention training reached *87 individuals*, up from *52* in Quarter 1, putting the cumulative figure at *56% of the Year 1 target* at the halfway point.

What did not

- Two sub-regions recorded no screening activity this quarter, both served by itinerant staff whose planned village travel did not occur.
- Referral-to-service linkage stands at *61%*. The federal indicator counts individuals *receiving* services after referral, so the 12 individuals without a recorded contact do not count toward the measure regardless of what was offered to them.

What we recommend before Quarter 3 closes

- **Close the linkage reporting gap.** Of the 12 unmatched referrals, our review of the referral log suggests most are a records problem rather than a service problem: the receiving provider has no obligation to report back. A one-page release and a 14-day follow-up call, added to the referral workflow, would convert most of these into reportable service contacts.
- **Schedule a make-up screening cycle** for the two sub-regions with no Quarter 2 activity, so the annual report does not show a coverage gap that reflects weather rather than program design.

1. Required performance indicators

These five indicators are reported quarterly to SAMHSA. Definitions and the reporting cadence are taken from the published FY2026 Garrett Lee Smith State/Tribal notice of funding opportunity; operational definitions used by this program are in Appendix A.

INDICATOR	Q1	Q2	CUMULATIVE	Y1 TARGET	% OF TARGET	STATUS
Individuals trained in mental health promotion	38	64	102	200	51%	ON TRACK
Individuals trained in suicide prevention	52	87	139	250	56%	ON TRACK
Individuals screened for suicidal ideation	0	214	214	600	36%	BEHIND PACE
Individuals referred to crisis or other mental health services	0	31	31	—	—	NO TARGET
Individuals receiving services following referral	0	19	19	—	—	ATTENTION

Table 1. Required quarterly indicators. All figures illustrative. Year 1 targets as stated in the approved work plan.

Progress against Year 1 targets, at the halfway point





Screening sits 14 points below elapsed time because collection began in month four rather than month one. At the Quarter 2 rate the Year 1 target is reachable; at the Quarter 2 rate *minus* the two inactive sub-regions, it is not.

2. Referral and service linkage

Indicator five counts individuals who *receive* services after referral, not individuals referred. The difference is the measure most often misreported in the first year of a grant, because the receiving provider is usually outside the grantee's record system and under no obligation to report back.

STEP	COUNT	RATE FROM PRIOR STEP	SOURCE OF RECORD
Screened for suicidal ideation	214	—	Screening instrument, program record
Screened positive, referral indicated	34	15.9%	Screening instrument, scored field
Referral made and logged	31	91.2%	Referral log
Service contact confirmed	19	61.3%	Provider confirmation, where returned
No confirmation on record	12	38.7%	Absence of a provider return

Table 2. Referral-to-service chain, Quarter 2. All figures illustrative.

> **How we would read this, and how we would not.** A 61% linkage rate is not evidence that 12 young people went unserved. It is evidence that 12 referrals have no return record. Those are different findings with different remedies, and conflating them would misstate the program's performance to its project officer in both directions — understating service delivery and overstating a care gap. We report the measure as the indicator defines it, and we report the records problem separately, because only one of them is fixable by the program this quarter.

ILLUSTRATIVE SAMPLE — PROGRAM FIGURES ARE NOT ACTUAL DATA

3. Reach across the region

The TCC region covers 235,000 square miles across six sub-regions — Yukon Koyukuk, Yukon Tanana, Lower Yukon, Upper Kuskokwim, Yukon Flats and Upper Tanana — and 42 member communities. Activity counts alone will always favor the road-accessible areas, so we report reach by sub-region every quarter and flag absence rather than letting it average out. In this sample the sub-regions are labelled A to F, because the activity figures are invented and no community should appear in a record of services it did not receive.

SUB-REGION	TRAINED	SCREENED	REFERRED	QUARTER 2 NOTE
Sub-region A	61	88	14	Road-accessible; two school sites
Sub-region B	34	52	8	Two village trips completed
Sub-region C	28	41	5	One trip completed, one deferred
Sub-region D	19	33	4	Single site
Sub-region E	9	0	0	NO SCREENING Planned travel did not occur
Sub-region F	0	0	0	NO ACTIVITY Planned travel did not occur

Table 3. Quarter 2 activity by sub-region. All figures illustrative; sub-regions are labelled A–F rather than named, because the activity shown is invented and should not be attributed to any community.

4. What the published data supports about program design

Program counts describe what the program did. Published data establishes what it is aimed at, and whether the aim is right. The figures below are **real**, drawn from the 2026 Alaska Native Injury Atlas. *Unlike the program figures elsewhere in this report, nothing in this section is illustrative.*

> **How we report regional data, and why.** The atlas publishes figures for thirteen regions. We report the Interior's own figures alongside the statewide picture, and **we do not rank regions against one another**. A comparative table invites reading one community's experience as a score against another's, which serves no purpose for this program and is not ours to publish. Where comparison aids interpretation, we give the statewide range.

MEASURE	VALUE	READING
Interior region	**15.9%**	About one in six Alaska Native injury hospitalizations from this region involved a suicide attempt
Median across the thirteen regions	16.9%	The Interior sits close to the middle of the statewide picture
Range across the thirteen regions	9.4% – 42.8%	Regional figures vary widely, for reasons the measure itself cannot separate

Table 4. Suicide attempt hospitalizations as a share of total injury hospitalizations, Alaska Native people, 2018–2023. Source: Alaska Native Injury Atlas, 4th ed., Part 2 (ANTHC, 2026), from Alaska Health Facilities Data Reporting Program data.

> **What this figure is, and the mistake it invites.** The denominator is *total injury hospitalizations in the region*, not regional population. So this is a share of injury burden, not a rate, and a region with fewer injuries of other kinds will show a higher percentage without any difference in suicide attempts at all. The atlas is explicit that it publishes no rates based on population at any geography.

A higher share can also reflect differences in identification and coding rather than differences in events. We would use this measure for what it supports: the share of regional injury burden involving suicide attempts, and its direction over time. We would not convert it into a population rate without a denominator we can name and source.

Age and sex distribution, statewide

The grant serves youth and young adults up to age 24. The statewide distribution is the evidence that this is the right population to serve.

AGE GROUP	FEMALE	MALE	TOTAL	SHARE OF ALL
10–19	334	94	**428**	**30.7%**
20–29	282	164	**446**	**32.0%**
30–39	168	104	272	19.5%
40–49	108	35	143	10.3%
50–59	38	28	66	4.7%
60+	26	12	38	2.7%
All ages	**956**	**437**	**1,393**	100%

Table 5. Suicide attempt hospitalizations by age and sex, Alaska Native people statewide, 2018–2023. Same source. Age groups follow the atlas's small-number restrictions.

The program is aimed where prevention reaches furthest. Ages 10 to 29 account for 62.7% of these hospitalizations statewide. The grant's up-to-24 population sits inside the age range where gatekeeper training and screening put the program in contact with the most young people — so the funded design is already pointed at the right place, and the evaluation's job is to show how far it reaches rather than to relitigate whom it serves.

Reaching young people where they already are. Young women are 78.0% of the 10–19 figures. Screening offered in settings young people already visit — school health, routine clinic visits, youth programming — reaches more of them than screening that waits for a referral. We report screening reach by sex and by setting each quarter so the Department can see whether its placement is working.

What the program adds. These figures describe young people who reached a hospital. Prevention works earlier than that — a trained adult who notices, a screening conversation, a referral that connects. Measuring that earlier contact is the whole purpose of the five indicators in Section 1, which is why this evaluation reports them as a connected chain rather than as separate counts.

Limits carried with these figures

CONSTRAINT	EFFECT ON USE
No population-based rates published, regionally or statewide	Counts and percentages only. No figure here is a rate, and none is presented as one
Inpatient admissions only, from Health Facilities Data Reporting	An attempt treated at a village clinic or an emergency department without admission does not appear. Material across 235,000 square miles where most communities have no hospital
Race recorded by the reporting facility; non-Tribal facilities may use single-race categories	Alaska Native counts are a known undercount of unknown size, particularly for multi-racial people — a floor, not a measurement
Data years 2018–2023; region assigned by home address	Predates the grant period. Establishes the baseline the program is measured against, not current-year performance

CONSTRAINT	EFFECT ON USE
Alaska YRBS most recent published cycle is 2023; the 2025 cycle has been fielded but not released	A new cycle will likely publish during this grant year and shift any baseline keyed to 2023. Re-verified each quarter

Table 6. Constraints on the published data above, as stated by the source.

5. Data quality and completeness

Every quarterly report states what is missing before it states what was found. A project officer who discovers a gap you did not disclose will discount the rest of the submission.

MEASURE	COMPLETE	MISSING	EFFECT ON THIS REPORT
Screening records with sub-region recorded	214 / 214	0	None
Screening records with age recorded	201 / 214	13	Age breakdown suppressed this quarter; 13 records would distort a small denominator
Referrals with a provider confirmation returned	19 / 31	12	Indicator five reported as 19. See Section 2
Training rosters with participant role recorded	151 / 151	0	None
Sub-regions reporting any activity	5 / 6	1	Sub-region F recorded none; its planned travel did not occur
Sub-regions reporting any screening	4 / 6	2	Regional screening totals are not regionally representative this quarter; stated wherever regional figures appear

Table 7. Quarter 2 data quality. All figures illustrative.

Changes to definitions or method since the last report

None this quarter. Any change to an operational definition is recorded here with the date it took effect and the figures affected, so a reader comparing quarters is never comparing two different measures under one name.

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6. Source register

Every figure in this report traces to a row in a delivered file. The register below is the index; the files themselves are delivered with the report in open, editable formats.

FIGURE IN THIS REPORT	DELIVERED FILE	RECORDS	EXTRACTED
Trained — mental health promotion (64)	q2_training_roster.csv	64	2027-04-02
Trained — suicide prevention (87)	q2_training_roster.csv	87	2027-04-02
Screened (214), screened positive (34)	q2_screening_records.csv	214	2027-04-02
Referred (31), service confirmed (19)	q2_referral_log.csv	31	2027-04-05
Sub-region distribution, Table 3	q2_subregion_rollup.xlsx	6	2027-04-05
External source metadata, Table 4	source_register.xlsx	3	2027-04-06

Table 8. Source register, Quarter 2.

Appendix A. Measurement framework

The operational definition behind each required indicator. This framework is produced in the first quarter of the engagement and is the document that makes every later report reproducible. Any change to it is versioned and disclosed.

INDICATOR	COUNTING UNIT AND RULE	SOURCE SYSTEM	INSTRUMENT	OWNER
Trained in mental health promotion	Distinct individuals completing a designated mental health promotion curriculum within the quarter. Completion, not attendance. An individual completing two curricula counts once per curriculum type, once per year	Training roster	Sign-in and completion sheet, with role and sub-region fields	Program staff; evaluator verifies
Trained in suicide prevention	Distinct individuals completing a designated gatekeeper or prevention curriculum within the quarter. Same completion and de-duplication rule	Training roster	Same instrument, curriculum coded	Program staff; evaluator verifies
Screened for suicidal ideation	Distinct individuals administered the designated screening instrument within the quarter, whether or not the result was positive. Re-screens of the same individual count once per quarter	Screening record	Designated screening instrument with scored field, age and sub-region	Clinical or program staff; evaluator verifies
Referred to crisis or other mental health services	Distinct individuals for whom a referral was made and logged following a positive screen or staff identification. Counted at the point of referral	Referral log	Referral form with destination provider and date	Program staff
Receiving services following referral	Distinct referred individuals with a confirmed service contact at the receiving provider. Requires a return record; an offer of service is not a service contact	Referral log, provider confirmation field	Provider confirmation, with release in place	Program staff; evaluator reconciles

Table A1. Operational definitions for the five required indicators.

Reporting calendar

DELIVERABLE	DUE	PREPARED BY
Quarterly performance submission	Within 30 days of quarter close	Evaluator, reviewed by Department Director
Quarterly evaluation report (this document)	Within 14 days of quarter close	Evaluator
Annual Programmatic Progress Report	Within 90 days of budget period close, on the approved template	Evaluator, submitted by grantee
Continuation application data package	At the grantee's application date	Evaluator

Table A2. Deliverables and dates.

ChiResearch Technologies, LLC · 6828 Flagstaff St, Hyattsville, MD 20785 · wilfred@chiresearchai.com · 410-929-2170
Illustrative sample prepared November 2026 to accompany a proposal for grant evaluation services. Program activity figures describe no real participants, services or outcomes. External sources are cited accurately and should be re-verified before use.